Meeting	Federation board, 14 January 2027
Agenda item	6
Paper	Choosing the 2027-28 deprescribing review
Prepared by	Colin French, Lead Clinical Pharmacist
Sponsor	Leigh Bond, Clinical Director
For	Decision

Run the benzodiazepine and Z-drug review in 2027-28

Recommendation

Run review B, benzodiazepines and Z-drugs in people aged 65 and over, across the member practices outside the Clun Valley network from April 2027. It would take about 290 patients off their medicine and avoid about 12 serious harms a year, 4 more than the runner-up, review C (gabapentinoids taken with an opioid, about 8). Review A (antipsychotics in people aged 65 and over) would avoid about 6.

The board is asked to approve review B for the medicines safety schedule of the 2027-28 contract with the integrated care board.

Review	Patients in scope	Started by a specialist service	Off their medicine at six months	Serious harms avoided a year
A Antipsychotics	612	86%	about 60	6
B Benzodiazepines and Z-drugs	1,239	6%	about 290	12
C Gabapentinoids with an opioid	1,636	44%	about 340	8

Source: the practices' issue, problem list and registration extracts at 31 December 2026; the trusts' medicine letters feed; the pilot outcome log. Patients are rounded to the nearest 10 and harms to whole numbers.

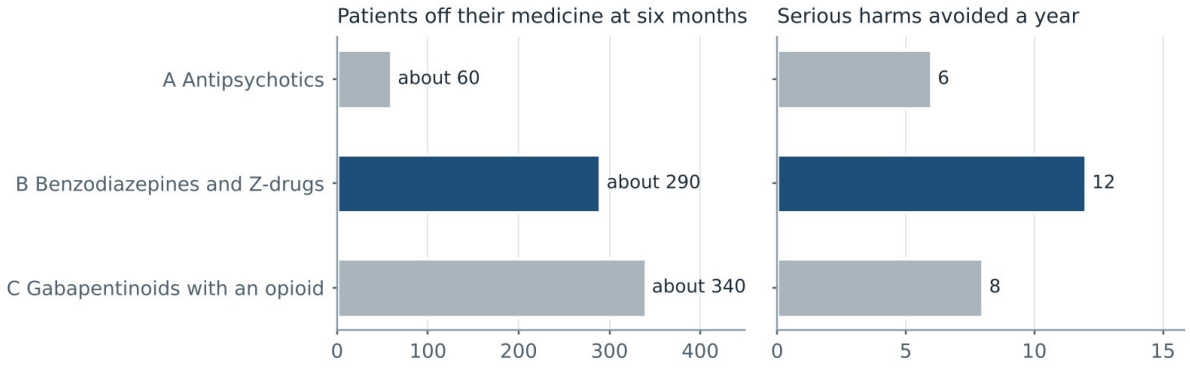


Figure 1. Review B avoids the most harm, although review C would take more patients off their medicine.

How the estimate was made

Patients in scope are those registered at a member practice outside the Clun Valley network who met the review's protocol on 31 December 2026, as the programme specification sets out (section 3). Each patient's chance of being off the medicine six months after review is taken from the Clun Valley pilot, and the harm it avoids from table 1 of the specification at their age on 31 December 2026.

The pilot's results turn on who started the medicine. Where a hospital or mental health trust service started it (a trust letter naming a medicine of the review's class in the 60 days before the practice first issued it), our pharmacists took 4 of 96 patients off; where the practice started it, 196 of 604. The Clun Valley practices are a long way from the memory and older adults' mental health services, so most of their antipsychotic patients were started by the practice. In the other networks a specialist service started 86% of the patients in scope for review A, against 6% for review B and 44% for review C. Each patient in scope is therefore given the pilot rate for their review and the way their medicine was started.

Pilot, off at six months	A Antipsychotics	B Benzodiazepines and Z-drugs	C Gabapentinoids with an opioid
Medicine started by the practice	63 of 136 (46%)	68 of 276 (25%)	65 of 192 (34%)
Medicine started by a specialist service	1 of 24 (4%)	1 of 24 (4%)	2 of 48 (4%)
All reviewed patients	64 of 160 (40%)	69 of 300 (23%)	67 of 240 (28%)

Source: deprescribing pilot outcome log, six-month checks; the trusts' medicine letters feed.

Applying each review's overall pilot rate to every patient instead would favour review A, at about 24 harms a year, because it treats the specialist-started antipsychotic patients as if they would come off as readily as the pilot's.

Note. Following section 3.3 of the specification, a review's patients off their medicine are the sum of each patient's chance, an expected number rather than a list of named patients.

Hospital medicine letters

Medicine letters from Wrekinside Teaching Hospitals and Marches Mental Health received by each network's practices, per 1,000 registered patients at the start of each half of 2026. A letter the mental health trust re-sent on 7 April 2026 is counted once, when it was first received.

Primary care network	January to June 2026	July to December 2026
Clun Valley PCN	46.0	43.3
Severn Gorge PCN	43.5	40.4
Teme Valley PCN	45.7	40.6
Wrekinside Central PCN	42.3	37.4
Wrekinside North PCN	43.0	41.7

Source: the trusts' medicine letters feed and the practices' document workflow log to 6 January 2027; registered list sizes on 1 January and 1 July 2026. From October the hospital trust's letters arrive through GP Connect and are matched to the workflow log by patient and the first receipt on or after the letter's date; a letter not received by 6 January 2027 is not counted.

The team's patient consultations by network and quarter of 2026 are in pharmacist_consultations_2026.csv.